

Ankle Fracture

Issuing institution: Worcestershire Acute Hospitals NHS Trust

Publication / update date: 2026-06-23

Original official URL: <https://www.worcsacute.nhs.uk/leaflets/ankle-fracture/>

Provenance note: This PDF is a local English-text snapshot of the official page. Navigation was removed.
The medical text below was not translated, paraphrased, or supplemented.

This leaflet will provide you with the information required to begin your rehabilitation following your ankle fracture.

Whilst your ankle has been immobilised in a cast or boot, the muscles and soft tissues may become stiff and weak. It is important to begin to exercise and strengthen your ankle in order to return to your previous level of function.

Your treatment will depend on how severe the injury is. Some ankle fractures can be managed without an operation using a cast or boot and may require a period of non-weight bearing. Some may require an operation to make the joint more stable. This usually requires a period of non-weight bearing whilst in a plaster cast and then progression to a boot. You will be advised by your physiotherapist at which stage you can begin the following exercises and when to progress.

Pain Pain is very normal following your fracture and it is important to manage this so you can complete your exercises regularly and be comfortable managing your daily tasks. Ensure you are taking the pain medication as prescribed by your Consultant. Speak to your GP or pharmacist if you are struggling with your pain levels.

Swelling You may notice swelling around the ankle joint and into the foot. Try using an ice pack on the area for 10-15 minutes maximum. You can use a packet of frozen peas or a gel pack. Ensure it is wrapped in a damp tea towel to protect your skin. Check regularly throughout application that you are not getting severe blanching (when the skin becomes pale or white) of the skin. If you get this, stop application immediately.

Smoking cessation Medical evidence suggests that smoking prolongs fracture-healing time. In extreme cases, it can stop healing altogether. It is important that you consider this information with relation to your recent injury. Stopping smoking during the healing phase of your fracture will help ensure optimal recovery from this injury.

Skin It is normal to experience dry and sensitive skin following removal of the cast. Wash the area with a non-perfumed soap and use non scented moisturiser such as E45 cream or a neutral oil to help moisturise the skin.

If you had an operation on your ankle, it is important to look after the condition of the scar. Once your wound is healed (unless otherwise advised) gently massage a non-perfumed moisturiser into your scar two to three times a day to hydrate the skin and make it supple. Massage is used to desensitise, relieve itching, stretch the scar, and move excessive fluid. When massaging the scar, you are advised to use your fingertip to help reduce swelling in a circular motion, slow but firm for approximately 20-30 times, more if the scar feels harder.

Walking Pattern Your walking pattern will be altered, and you may still require a walking aid to use when you are walking. This will help with healing, managing pain and help your balance. Your physiotherapist will guide you through progressing to walking with no aid. Please see our leaflet 'Walking with Crutches' and the accompanying video for further information.

Exercises The following exercises are split into early stage and mid stage. The early stage exercises can be started once you have had your cast removed. These aim to improve range of movement and strength within the ankle. They then progress to further strengthening and balance exercises.

Your physiotherapist will highlight which exercises you should be completing and they will fill in below how often you should be completing them for. Your physiotherapist will also advise what colour exercise band you should use.

Early Exercises

Ankle circles Move ankle round in a circle. Both clockwise and anti clockwise.

Dorsiflexion to plantarflexion Point your foot up and down within a comfortable range of movement.

Inversion to eversion Keep heels on floor and hip and knee still. Bring toes in towards each other, then go the opposite way and bring foot up and out to the side.

Towel stretch calf Loop towel around bottom of foot and draw slowly towards you, feeling a stretch along the back of your calf. Hold for 20-30 seconds.

Towel stretch in and out Loop towel around bottom of foot near your toes. Move toes in and out, using the towel to help you do so.

Toe curls Place towel underneath foot with foot flat on floor. Curl toes trying to scrunch the towel up.

Seated heel raise In sitting, have feet flat on floor. Lift heels up off floor as high as able, keeping toes in contact with the floor. Slowly lower down. Then lift up toes off the floor, keeping heel on the floor.

Mid Stage Exercises

Resisted dorsiflexion Hook an exercise band around top of foot. Bring foot up towards you then lower down.

Resisted plantarflexion Hook an exercise band underneath foot. Push down in to resistance.

Resisted inversion Loop an exercise band around a sturdy object or have someone hold it. Place something under heel. Alternatively, hook over other foot and cross leg over other leg. Keep heel still and curl inside border of the foot in.

Resisted eversion Loop an exercise band around both feet. Keep heel still and bring toes out to side.

Heel raises Use a wall or hard surface for support if needed. Rise up on to tip toes as high as able. Hold for 3 seconds, then slowly lower down.

Single leg balance Begin by using a wall or surface to support. Stand on one leg and aim for 30 seconds. Once able to do this, practise standing on one leg without holding on to a surface.

Squat Stand with feet hip width apart. Squat down as far as feels comfortable, then push back up again. If ankle feels stiff, place a rolled up towel under heel to make it easier.